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THEORETICAL ARTICLE

Meaning in Life as a Transdiagnostic Factor

Conceptual Clarifications, Theoretical Models, and Open Questions

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ABSTRACT

Transdiagnostic approaches investigate psychological characteristics and processes that contribute to the onset and maintenance of psychological problems across diagnostic categories. This theoretical paper examines whether, and in what sense, meaning in life can be conceptualized as a transdiagnostic factor. Drawing on criteria discussed in the transdiagnostic literature, the paper first clarifies the concept of meaning in life, distinguishing between presence of meaning and search for meaning as well as between the components of coherence, purpose, and significance. Three models are then contrasted: meaning in life as a vulnerability or protective factor, as an outcome of successful meaning making after stressful events, and as an expression of overarching motivational structures. It is argued that a lack of experienced meaning should be understood less as a distinct mechanism than as a nodal point at which motivational, cognitive, and emotional processes converge. Finally, implications for assessment and psychotherapy are discussed, and methodological requirements for an empirical test of the transdiagnostic status of meaning in life are outlined, in particular longitudinal designs, the separation of conceptually overlapping constructs, and attention to cultural variability.

Keywords: meaning in life · transdiagnostic approach · psychopathology · meaning making · protective factors

ZUSAMMENFASSUNG

Transdiagnostische Ansätze untersuchen psychologische Merkmale und Prozesse, die über verschiedene Störungsbilder hinweg an Entstehung und Aufrechterhaltung psychischer Beschwerden beteiligt sind. Der vorliegende theoretische Beitrag prüft, ob und in welchem Sinne Lebenssinn als transdiagnostischer Faktor konzipiert werden kann. Ausgehend von den Kriterien, die in der transdiagnostischen Literatur für entsprechende Konstrukte diskutiert werden, wird zunächst der Begriff des Lebenssinns präzisiert. Unterschieden werden das Vorhandensein von Sinn, die Suche nach Sinn sowie die Komponenten Kohärenz, Zielorientierung und Bedeutsamkeit. Anschließend werden drei Modellvorstellungen gegenübergestellt: Lebenssinn als Vulnerabilitäts- bzw. Schutzfaktor, als Ergebnis gelingender Bedeutungsbildung nach belastenden Ereignissen und als Ausdruck übergreifender Motivationsstrukturen. Es wird argumentiert, dass ein Mangel an erlebtem Sinn weniger als eigenständiger Mechanismus denn als Knotenpunkt verstanden werden sollte, an dem motivationale, kognitive und emotionale Prozesse zusammenlaufen. Abschließend werden Implikationen für Diagnostik und Psychotherapie diskutiert sowie methodische Anforderungen formuliert, die für eine empirische Prüfung des transdiagnostischen Status von Lebenssinn erforderlich wären, insbesondere längsschnittliche Designs, die Trennung konzeptuell überlappender Konstrukte und die Berücksichtigung kultureller Variabilität.

Schlüsselwörter: Lebenssinn · Transdiagnostik · Psychopathologie · Bedeutungsbildung · Schutzfaktoren

1 Introduction

The categorical classification of mental disorders has proven indispensable for communication, health care, and research. At the same time, its limitations have long been known: high comorbidity rates, blurred boundaries between diagnoses, and the observation that similar psychological processes are at work across different disorders have led to a transdiagnostic perspective. In an influential monograph, Harvey et al. (2004) showed that processes such as selective attention, repetitive negative thinking, or avoidance behavior occur across numerous disorders. At the structural level, it has also been discussed whether a general psychopathology factor can be identified (Caspi et al., 2014).

Against this background, it is natural to ask whether meaning in life – or its absence – can also be understood as a transdiagnostic factor. Logotherapy anticipated such an assumption in a certain sense by describing the frustration of the will to meaning as a

condition that can manifest itself in different symptoms (Frankl, 1969). The present contribution examines this intuition in the light of current theoretical criteria. It is intended as a conceptual analysis and not as a summary of original empirical data.

2 Criteria for Transdiagnostic Constructs

Nolen-Hoeksema and Watkins (2011) proposed a heuristic by which transdiagnostic models can be specified. They distinguish distal risk factors that increase vulnerability, proximal processes that are directly involved in symptom formation, and moderating conditions that explain why a common factor leads to different disorders in different individuals. Accordingly, a construct does not deserve the label "transdiagnostic" merely because it correlates with several disorders. What is required, rather, is a theoretically grounded model of how it affects the development and maintenance of symptoms.

For meaning in life, this gives rise to a twofold task. First, it must be clarified what exactly is understood by meaning in life. Second, it must be determined at which level of the model – as a distal factor, as a proximal process, or as a moderator – it is to be located. The two questions are not always kept sufficiently apart in the literature.

3 Conceptual Specification

With the distinction between the presence of meaning and the search for meaning, Steger et al. (2006) introduced a differentiation with far-reaching consequences for empirical research. While the experienced presence of meaning is generally associated with well-being, the relationship between the search for meaning and mental health is more complex and apparently context-dependent. Martela and Steger (2016) further distinguished three facets of the experience of meaning: coherence in the sense of the comprehensibility of one's own life, purpose in the sense of a motivating orientation toward valued goals, and significance in the sense of experiencing one's own life as valuable and as mattering.

Schnell (2009) has also shown that a distinction should be made between the sources from which people derive meaning and the global experience of meaning. This distinction is clinically significant: a lack of experienced meaning can be due to the loss of individual sources of meaning, to a low diversity of such sources, or to a lack of integration into an overarching self-understanding. Furthermore, Schnell (2020) has drawn attention to the state of "existential indifference," in which meaning is neither experienced nor sought – a state that differs clearly from an acute crisis of meaning.

For the transdiagnostic question, it follows that "meaning in life" is not a unitary construct. Anyone who speaks of meaning as a transdiagnostic factor must specify which facet is meant, since coherence, purpose, and significance are potentially linked to different psychopathological processes.

In addition, the experience of meaning and the search for meaning should not be understood as opposite poles of a single dimension (Steger et al., 2006). A person may experience their life as meaningful and at the same time search for deeper meaning; conversely, someone may experience little meaning without actively searching for it. These constellations presumably have different clinical significance, which must be taken into account when formulating transdiagnostic hypotheses.

4 Three Model Conceptions

A first model understands experienced meaning as a protective factor and its lack as a vulnerability factor. This view is related to salutogenic considerations such as those formulated by Antonovsky (1987) with the concept of the sense of coherence, in which meaningfulness occupies a central position as the motivational component. In this model, meaning in life would be a distal factor in the sense of Nolen-Hoeksema and Watkins (2011) that influences the probability of various disorders without determining their specific form.

A second model emphasizes the process dimension. In her meaning-making model, Park (2010) describes how stressful events create a discrepancy between global meaning structures and the situational appraisal of an event, which can be reduced through processes of meaning making. Within this framework, psychological complaints could be understood as an expression of unsuccessful meaning making. Here, meaning is not primarily a stable trait but the result of ongoing processing that may be proximally involved in the maintenance of symptoms.

Finally, a third model interprets the experience of meaning as an expression of overarching motivational structures. Baumeister (1991) described needs for meaning relating to purpose, values, efficacy, and self-worth. From this perspective, a lack of meaning points to a disturbance of motivational orientation, which may manifest itself, for example, in reduced drive, avoidance behavior, or compensatory strategies. Yalom (1980), too, described meaninglessness as one

of the ultimate concerns of existence, with which people deal in very different, sometimes dysfunctional ways.

Logotherapy cannot be clearly assigned to any of these models. Frankl's concept of noogenic neurosis describes complaints that arise primarily from an existential conflict or from existential frustration (Frankl, 1969). It thereby assumes a specific etiology that is supposed to differ from psychogenic and somatogenic disorders. At the same time, Frankl emphasizes that an existential vacuum can also exist without pathological significance and can manifest itself in different forms. From a transdiagnostic point of view, this dual function is remarkable because it conceptualizes the frustration of meaning both as a specific cause and as a nonspecific background factor.

5 Meaning as a Nodal Point: An Integrative Interpretation

The three models are not mutually exclusive. There is much to be said for understanding a lack of experienced meaning less as an independent, isolable mechanism than as a nodal point at which motivational, cognitive, and emotional processes converge. Findings according to which positive affect influences the momentary experience of meaning (King et al., 2006) illustrate that the experience of meaning is not independent of a person's affective state. Conversely, it is plausible that a stable value orientation can buffer affective crises.

Such a nodal-point interpretation has consequences for research. It warns against treating the experience of meaning as a mere reflection of depressive symptoms, as well as against inflating it into an all-encompassing explanatory principle. Heintzelman and King (2014) have also pointed out that most people describe their lives as fairly meaningful. A clinically relevant lack of meaning is therefore the exception rather than the rule, which is significant for determining cut-off values and risk groups.

A further aspect concerns the direction of the associations. Cross-sectional correlations between a low experience of meaning and psychological complaints are compatible with several causal assumptions: a lack of meaning may promote complaints, complaints may impair the experience of meaning, or both may be due to common third factors. A transdiagnostic model must explicitly distinguish these possibilities and must not prematurely infer directions of effect from associations.

6 Implications and Open Questions

For diagnostic assessment, the analysis suggests assessing the experience of meaning in a facet-specific and cross-disorder manner rather than merely recording it as a concomitant variable. For psychotherapy, the question arises whether meaning-centered interventions can be used as supplementary modules in different treatment contexts. This would correspond to Frankl's self-understanding of logotherapy as a supplement to other procedures.

An empirical test of the transdiagnostic status of meaning in life would require, in particular, longitudinal designs that capture temporal sequences between the experience of meaning and symptom development, as well as a careful separation of conceptually overlapping constructs such as hopelessness, well-being, or self-efficacy, and consideration of cultural variability in the significance of meaning. As long as these requirements are not met, talk of meaning in life as a transdiagnostic factor should be understood as a heuristically fruitful hypothesis, not as an established finding.

For therapeutic practice, it should also be borne in mind that addressing meaning is not appropriate at every point in a treatment. In acute crises, with severe depressive symptoms, or with pronounced reduction in drive, the explicit question of the meaning of one's own life can be overwhelming or intensify feelings of failure. A transdiagnostic understanding of meaning therefore does not argue for an undifferentiated application of meaning-centered techniques but for careful determination of the indication that takes into account the person's condition, their resources, and the course of treatment.

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